Pediatric Health History

Pediatric Health History-PRACTICAL

Third Stage

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Pediatric Nursing

OBJECTIVES

1. Define health history
2. Numerate and describe types of Pediatric Health Histories
3. Differentiate between objective and subjective data.
4. Describe Methods of Data Collection.
5. Identify How to Conduct a Pediatric History Interview.

Pediatric Health History

A health history is a collection of subjective data that provides a detailed profile of the patient's health status.

Nurses use therapeutic communication skills and interviewing techniques during the health history to establish an effective nurse–patient relationship and to gather data to identify actual and potential health problems.

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Types of Pediatric Health Histories

1. Initial (comprehensive) health history:

An initial health care visit requires a comprehensive health history, which creates a database of information for the child who is new to the provider's practice. The data collected in this history focus not just on past or current illnesses, but also on wellness, health patterns, health knowledge, and activities of daily living (ADL).

Types of Pediatric Health Histories

3. Focused (episodic) health history:

This history is problem-centered, focusing on the present illness or problem. A focused health history is all that is necessary when the child presents with an acute problem or the exacerbation of a chronic condition.

Types of Pediatric Health Histories

4. Follow-up health history:

When the child returns to the office after an illness or injury, a follow-up

history is taken to assess whether the problem is the same, better, or worse.

Types of Pediatric Health Histories

5. Emergency health history:

An emergency health history involves the quick collection of data in

an acute situation. While this history resembles a focused history, it

needs to be obtained rapidly, usually simultaneously with

interventions, such as the administration of medications or respiratory treatments.

Data Collection

1. Subjective data:

are information perceived only by the affected person; these data cannot be perceived or verified by another person.

DATA COLLECTION

2. Objective data:

are observable and measurable data that can be seen, heard, or felt by someone other than the person experiencing them.

Methods of Data Collection

1. Observation

- Notes the **general appearance** and **behavior** of the client
- Helps to determine the **client's status**, both **physical** and **mental**

2. Interview

- Preparation
- Stages
  - Introduction
  - Working
  - Closure

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Methods of Data Collection

4. Physical examination

- **Assessment techniques**
  - Inspection
  - Palpation
  - Percussion
  - Auscultation

5. Laboratory and diagnostic data

How to Conduct a Pediatric History Interview

Obtaining an accurate and complete pediatric health history is essential when providing health care to children.

The pediatric health care provider must possess excellent communication skills when working with children and families in order to elicit the most accurate and complete health history while maintaining provider– patient trust.

Pediatric Health History

- **For infants and preverbal children**, the primary source of historical information is the parent or primary caregiver.
- **For older infants and toddlers**, it is important that the parent remain within eyesight of the child to minimize stranger anxiety.

Pediatric Health History

- Some preschoolers can answer questions during the history, depending on their language abilities, developmental level, and temperament.
- Young children who are very shy should not be forced to participate in the interview, and children should not be overwhelmed with too many questions..

Pediatric Health History

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- **Children who are school-aged and older** can provide their own health history with verification from the parent or caregiver as needed.

Older school-aged children and adolescents have a heightened need for privacy..

Age-specific interview and assessment tips

Infant





- Before performing a procedure, talk to and touch the infant.
- Use a gentle touch.
- Speak softly.
- Allow the infant to hold a favorite toy during the assessment.

Age-specific interview and assessment tips.

Toddler

- Encourage the parents to be with you during the interview.
- Allow the toddler to be close to his parents.
- Provide simple explanations and use simple language.

Age-specific interview and assessment tips.

Preschool child

- Ask simple questions.
- Allow the child to ask questions.
- Provide simple explanations.
- Avoid using words that sound threatening or have double meanings.

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Age-specific interview and assessment tips.

School-age child

- Provide explanations for procedures.
- Explain the purpose of equipment, such as an ophthalmoscope to see inside the eye.

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Age-specific interview and assessment tips.

Adolescent

- Give the adolescent control whenever possible.
- Facilitate trust and stress confidentiality.
- Encourage honest and open communication.
- Be nonjudgmental.

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Subjective Data



Subjective Data

A. Biographical data (demographical data) (ID)

1. **Name**
2. **Age**
3. **Gender** (male-or-female)
4. **Marital status** (married, single, divorce)
5. **Educational level** (primary, secondary, diploma,....)
6. **Occupation** (worker, officer, gainer)
7. **Religion** (Muslimism, Jewish, Christian
8. **Birth date**
9. **Birth place**
10. **Phone number**
11. **Phone number of significant person**
12. **Address**

Subjective Data

B. Present history (Chief Complaint (CC))

1. **C: Characteristics**
2. **O: Onset**
3. **L: Location + Radiation**
4. **S: Severity** (0-10 scale)

0-4 mild\ 5-6 moderate\ 7-10 sever

5. **D: Duration**
6. **P: Pattern** ↑↓
7. **A: Association sign and symptom**

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Subjective Data

C. Past Medical History

• Prenatal, Birth, and Neonatal History •

The depth of this history-taking depends on the child's age. The younger the child, the more important this information is, as prenatal, birth, and neonatal events are crucial to a child's future physical and psychosocial growth and development and may explain any neurologic or developmental problems.

TABLE 35–2 ✣ Birth History

Prenatal	■ Mother's age, health during pregnancy, prenatal care, weight gained, special diet, expected date of birth ■ Details of illnesses, x-ray findings, hospitalizations, medications, complications, and timing during pregnancy ■ Prior obstetric history
Antenatal—description of birth	■ Site of birth (hospital, home, birthing center) ■ Labor induced or spontaneous, length of labor, time/duration or rupture of membranes ■ Vaginal or cesarean birth, forceps or suction used, vertex or breech position ■ Gestation at birth, single or multiple birth
Condition of baby at birth	■ Weight, Apgar score, cried immediately ■ Need for incubator, oxygen, suctioning, ventilator ■ Any abnormalities detected, meconium staining
Postnatal	■ Difficulties in the nursery—feeding, respiratory difficulties, jaundice, cyanosis, rashes ■ Length of hospital stay, special nursery, home with mother ■ Breast- or bottle-fed, weight lost/gained in hospital ■ Medical care needed in first week—readmission to hospital

• Acute and Chronic Illnesses •

This component of the history comprises the child's **current health status.**

Acute Illnesses
respiratory infections
ear infections
diarrhea
Vomiting
dehydration.

Chronic Illnesses
congenital anomalies, genetic disorders, cancer, rheumatic fever, heart disease, hypertension, obesity, diabetes, sickle cell disease, cystic fibrosis, asthma, allergies, eczema, seizure disorders, neurologic infections, tuberculosis, renal disease, depression, and anxiety disorders

- **Childhood Infectious Diseases** -

The provider should determine if the child has had **measles, mumps, rubella, varicella, pertussis, streptococcal infections**, or has recently been exposed to these illnesses

Allergies •

All allergies to **medications, food, animals, and insect bites; environmental allergies; and a history of eczema, allergic rhinitis, contact dermatitis.**

Medications •

The provider should ask about all **current prescribed** or over-the-counter (**OTC**) medications that the child is taking.

Hospitalizations •

The date, the child's age, and the reason for any hospitalization as well as the name of the hospital and duration of the stay should be recorded.

• **Surgeries** •

The date of any surgical procedure should be recorded,

• **Injuries** •

The date of any injury should be documented in addition to the age of the child when injured.

Immunizations •

The child's immunization history should include which immunizations were received and at what age.

D. Family Medical History

Obtaining the child's family health history is **necessary** to determine the health status of immediate family members and to identify and document **any inherited disorders** that the child may have.

Pain Assessment

- This assessment should include a pain intensity measure. the **F**ace **L**egs **A**ctivity **C**ry **C**onsolability (**FLACC**) scale could be used.

CRITERIA	SCORE 0	SCORE 1	SCORE 2
Face	No particular expression or smile	Occasional grimace or frown; withdrawn, disinterested	Frequent to constant frown, clenched jaw; quivering chin
Legs	Normal position or relaxed	Uneasy, restless, tense	Kicking or legs drawn up
Activity	Lying quietly, normal position, moves easily	Squirming, shifting back and forth, tense	Arched, rigid, or jerking
Cry	No crying (awake or asleep)	Moans or whimpers, occasional complaint	Crying steadily, screams or sobs; frequent complaints
Consolability	Content, relaxed	Reassured by occasional touching, hugging, or being talked to; distractible	Difficult to console or comfort

Interpreting the Behavioral Score
Each category is scored on the 0–2 scale, which results in a total score of 0–10.
0 = Relaxed and comfortable; 1–3 = Mild discomfort; 4–6 = Moderate pain; 7–10 = Severe discomfort or pain or both.

Source: Merkel, Voepel-Lewis, Shayevitz, and Malviya (1997).

Nutrition

- This is an important area to assess during **both well and illness visits**. Pediatric nutritional assessment must be tailored to the **child's age.**

Infants.

The provider should ask if the infant is **breastfed or bottle-fed** and how **frequently**.

The provider should note whether breastfed infants are supplemented with formula. For infants who are taking formula, the provider should determine the **type of formula** and **how many ounces** of formula the infant consumes per day.

Toddlers.

- The provider should ask if the toddler drinks cow's milk and, if so, what type (e.g., whole vs. skim) and how many ounces a day.

Preschoolers and School-Aged Children.

The Provider should ask the child and parent to describe a **typical meal**, the child's **favorite food**, and a **typical snack**.

Adolescents. Providers should ask adolescents to describe **typical meals** and **snacks** and the **frequency** with which they consume **fast food and soda**. With the parent out of the room, providers should ask adolescent patients if they are satisfied with their body image or if they view themselves as fat.

Physical Activity

The provider should ask about the **number of hours** the child spends each day watching television, playing video games, or using the computer.

Elimination

- **Newborns and Infants**. For newborns, the provider should ask about number of wet diapers a day (**six to eight are normal**),.
- **Toddlers and Preschoolers**. Renal function is not fully developed until **age 2 years**. Toddlers and preschoolers urinate **8 to 14** times a day ,the number of stools a day varies, according to the child's diet; **one to three stools** a day is usual but **one stool every 3 to 4 days** is within normal limits.

School-Aged Children. School-aged children should be **fully toilet trained**. The provider should continue to ask about the number of voids a day (**usually five to six, depending on fluid intake**) and whether the child remains dry through the night (**nocturnal enuresis is within normal limits until the age of 5–6 years**). The frequency of stools should be assessed (normal ranges from **one to three** a day, to **one stool every 2–3 days**).

Adolescents: It is also important to ask about urinary frequency or urgency, which may **indicate either urinary tract or sexually transmitted infections**. **Polyuria** in this age group can be a sign of **type 1 diabetes**. Frequent or **chronic diarrhea** may indicate **laxative use**.

Safety

assessing the safety of the environment of an infant, child, or adolescent is extremely important, and the areas of assessment vary according to age and developmental level.

Sleep

- infants should be placed in the **supine position** for naps and at bedtime because of the association of sudden infant death syndrome (**SIDS**) with a prone sleep position.

Sleep Needs of Infants and Children

Birth–6 weeks	Approximately 16.5 hours a day, half of which is daytime sleep; sleeps 2–4 hours at a time
1 month	15.5 hours a day; may sleep 4–6 hours at a time
3 months	15 hours a day; bulk of sleep shifts to nighttime hours
4 months	Begins to nap at regular times during day; may sleep 7–8 hours at a stretch
6 months	14.25 hours a day, including two naps
9 months	14 hours a day, including two naps
12 months	13.75 hours, including one to two naps a day (The morning nap is given up between 12 and 24 months of age. The afternoon nap persists until approximately age 4–5 years.)
18 months	13.5 hours a day, including one nap
2 years	11–12 hours at night, one nap a day lasting 1–2 hours
3 years	12 hours a day total, including one 1-hour nap during the day

4 years	11.5–12 hours of sleep a day; many 4-year-olds stop naps, needing an earlier bedtime to get required sleep
5 years	11 hours, with most children stopping naps at this age
6 years	10.5–11 hours a day
7 years	10.5–11 hours a day
8 years	10.25–10.75 hours a day
9 years	10 hours a day
10–12 years	9.25–10 hours a day
Adolescents	8–9 hours a day

Common Pediatric Sleep Problems

SLEEP PROBLEM	AGE GROUP	CLINICAL FINDINGS	ASSESSMENTS
Sleep refusal	Toddler/ preschooler	Child refuses to settle once put to bed	Do parents maintain a consistent sleep routine? Are security objects used? Do parents reinforce calling out or crying behaviors?
Night waking	Any age	Child needs help during night to return back to sleep	Do parents put child to bed while he or she is still awake so that child becomes accustomed to falling asleep independently? Do parents rock child to sleep in their arms and then transfer child to bed? Do parents reinforce calling out or crying behaviors? Are security objects used?
Trained night feeder	Infants	Infant older than 4 months awakens, predictably, wanting to be fed	Do parents put child to bed with a bottle? Have parents tried to end this bedtime ritual either by gradually discontinuing night feedings or by discontinuing night feedings completely?
Delayed sleep phase	Any age	Child goes to bed late and awakens late	Have parents tried awakening child 15 minutes earlier each day until appropriate bedtimes and waking times result?
Advanced sleep phase	Any age	Child goes to bed early and awakens early	Have parents tried putting child to bed later so that awakening will occur later?
Unpredictable schedule	Any age	Child goes to bed and awakens at random times	Have parents maintained a predictable family schedule and consistent expectations for bedtime?

Nightmares	Most common in preschoolers; occur from 18 months through adolescence	Child awakens crying and fearful with memory of dream, which usually occurs in the latter half of night; child is slow to return to sleep	Do parents comfort and reassure child? Is a nightlight or closet light used to help child who is afraid of the dark?
Night terrors and sleepwalking	Most common in preschoolers; occur from 18 months through adolescence	Child begins screaming or crying but is not fully awake; child usually has no memory of event; night terrors usually occur in first third of the night; sleepwalkers arise and walk about without being fully awake	Do parents try to protect child from injury? Do parents help child to lie down and return to sleep? Do parents of sleepwalkers use gates at the top of stairs?
Insomnia/ hypersomnia	School age and older	Child is unable to sleep or excessive sleeping, which may be related to depression	Do parents ensure that child has no stimulating activities at bedtime that interfere with sleep such as caffeine consumption, rough play, or violent and upsetting television, movies, or other media? Has child been medically evaluated for hypersomnia to rule out medical conditions such as severe anemia, hypothyroidism, or depression?
Obstructive sleep apnea	Preschoolers and older	Child sleeps restlessly, with snoring, periods of respiratory distress, or apnea while sleeping and exhibits daytime fatigue	Has child been referred for sleep studies, allergy consults, or possible adenoidectomy?

E. Social History

- Family.
- Home Environment.
- Economic Status.
- Immigrant Status.
- Emergency Room Use.

Growth and Development

A complete history of a child's growth and development entails determining achievement of developmental milestones in the areas of physical growth, gross and fine motor development, cognitive abilities, language development, social interaction, behavior, and temperament.

- Gross motor milestones.
- Fine motor milestones.
- Language milestones.
- Psychosocial development.

Gross motor milestones can be assessed by asking about infant **head control**, rolling over, sitting and standing without support, crawling, walking, and dressing.

Fine motor milestones can be assessed by exploring areas such as reaching for objects, achievement of a pincer grasp, self-feeding.

Language milestones can be assessed by asking about when the child spoke his or her first word with meaning, combined two to three words, and spoke in sentences.

Psychosocial development can be assessed by asking the parent about the child's temperament, play, attachment to and ability to separate from the primary caregiver, self-care abilities, peer relationships, and school performance.

F. Spiritual History

It is important for the health care provider to conduct a spiritual assessment as part of the initial health history.

Review of systems:

Review of systems	Done to establish the past and present health state of each body system • General • Integument • Head • Eyes • Ears • Nose • Mouth or throat • Neck • Chest • Lymphatic • Respiratory • Cardiovascular • Gastrointestinal • Genitourinary • Gynecologic • Musculoskeletal • Neurologic • Endocrine • Hematologic • Psychosocial

Thank you



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